

Business Case Intake — Interview Summary

Project: SyncMD AI Ambient Notes Module

Date: May 25, 2026

Interviewee: Jorge Sanchez Jr. (SFP Operator / SyncMD Lead)

Project type: SyncMD platform module — internal SFP deployment + off-site PCP coverage

Completeness score: 12–13 / 13 → BC can be generated immediately

BLOCK 1 — THE PROBLEM

1.1 Problem: Delays in notes + quality control issues as we scale. Need to centralize provider notes. Need all notes V28 compliant and maximizing CPT codes for referring PCPs. Need to control sleep study denials. Human error compounding at scale.

1.2 Who it hurts: All parties — referring PCPs/patients, PRM (billing co), SFP (providers/ops/billing).

1.3 Specific example: Referring PCPs demand faster note turnaround. AI ambient delivers a finished note before the patient leaves the room — multi-day delay → real-time delivery.

1.4 Duration: Day 1 of the practice — 30 years. Structural, worsening with scale.

Stakeholder quote:

"Our referring PCPs want notes faster and faster — AI ambient listening will have the note finished before the patient walks out of the room. And internally, JSM wants this established — he's done hearing the noise from providers who aren't happy with the current notes."

— Jorge Sanchez Jr., SFP Operator (relaying Dr. JSM, physician-owner)

BLOCK 2 — WHY NOW

2.1 Trigger: 8 months of recurring conversations with JSM — kicked off SFP's broader tech push. Catalyzing event: one specific provider creating internal noise around note-signing delays and excessive time per patient.

2.2 Cost of 90-day inaction:

- Provider friction continues
- JSM doesn't see full investment in technology platform
- Sleep study denials continue to spike

2.3 External deadline: None. Internal pressure only.

2.4 Cost of waiting (per month):

- MA overtime: ~80 hrs / 2 wks (~40 hrs/wk) → **~\$7–8K/mo, ~\$90–95K/yr**
- New EMR rollout amplifies the speed gap from missing AI notes
- Provider replacement risk: ~\$50K if noisy provider walks
- Sleep denial spike continues (~\$15K/yr baseline)

BLOCK 3 — IMPACT (NUMBERS)

3.1 Financial:

Impact	Annual \$
Sleep study denials recovered	~\$15K
MA overtime eliminated	~\$93K
Provider retention (avoided cost)	~\$50K
Client retention risk (PCP referrals)	TBD

Impact	Annual \$
V28/HCC compliance	No direct \$ — competitive moat only
Floor estimate	~\$158K/yr

3.2 Operational:

- Scope: whole organization — ~8 providers, MAs across 5 clinics, sleep techs across 3 labs, billing, front desk
- MA OT burden: ~40 hrs/wk
- ≥1 FTE equivalent burned on note rework

3.3 PCP awareness: Internal only — no formal escalations. Note speed is a latent competitive opportunity, not yet a complaint vector.

3.4 Data sources:

- ADP payroll → MA OT hours
- Sleep study denial report → denial \$ baseline
- Provider note completion dashboard → signing lag baseline
- HCC V28 coding data → coding capture baseline
- Gap: PCP-side metrics on note turnaround perception not measured

BLOCK 4 — THE SOLUTION

4.1 Final state vision: Provider walks into the room → patient and doctor interact normally → AI ambient listens, filters small talk, captures only the medical content → note is drafted in real-time with proper structure → brings volume and depth currently missing → adds value (V28 coding, HCC capture, billing-ready). MAs out of the loop.

4.2 Deliverables:

1. AI ambient scribe deployed across all SFP exam rooms (5 clinics)
2. Notes signed same-day (vs. multi-day lag)
3. V28/HCC code suggestions auto-flagged in every note
4. Sleep study notes auto-validated for denial-prone missing elements
5. Centralized note QC dashboard (PRM-visible)

4.3 Approach: Need help selecting vendor. **Critical requirement:** must integrate as a module behind the SyncMD dashboard — no separate platform, no additional login. Providers stay in one workflow.

4.4 Prior attempts: Human scribes tried ~5 years ago — didn't work out.

4.5 Options Analysis:

- **Option A — Do nothing:** Cost of inaction ~\$158K+/yr floor.
- **Option B (Preferred) — Third-party AI engine integrated into SyncMD dashboard:** Fastest deploy, leverages mature ambient AI vendors via API, SyncMD owns UX/integration. Lower upfront cost.
- **Option C (Considered, rejected) — Build AI engine in-house at SyncMD:** Full IP control but massive capex, multi-year R&D, ML talent required, opportunity cost vs. mature vendors. Not viable on current timeline.
- **Why B:** Speed-to-value, capital efficiency, SyncMD captures workflow/integration moat without trying to out-engineer mature ambient AI vendors.

6-month vision: Internal noise cleaner. Notes more compliant. SFP positioned as a cutting-edge pulmonary group with proprietary in-house dashboard and working ambient notes — competitive moat and PE-readiness signal.

BLOCK 5 — SCOPE

Out of scope:

- Building NEW platforms (only build inside SyncMD)
- Hiring / org restructuring

Architecture: All work runs under **SyncMD as master platform**. SyncMD = parallel dashboard to CureMD EMR (not a replacement). AI ambient = module 1. AI sleep scoring = module 2. Future modules stack into same platform.

Strategic positioning value: Adds prestige — when a PCP refers a patient, notes are available immediately. Reinforces SFP-as-premium-specialist positioning.

Rollout approach: Phased pilot. Start at smaller sites → roll out site-by-site to all 5 clinics.

BLOCK 6 — OKRs

#	Objective	Key Result	Baseline	Target	By when	Owner
1	Eliminate MA OT tied to notes	MA OT hrs / 2 wks	80 hrs	<10 hrs	Immediately at go-live	Management
2	Cut note turnaround time	Encounter → signed notes	7BD (dashboard)	Same-day	Immediately at go-live	Provider champion
3	Reduce sleep study denials	Denial \$ / qtr	~\$3.75K/qtr	<\$1K/qtr	Within 1 mo	PRM (Oriana group)
4	Max V28/HCC coding capture	% notes w/ HCC capture	7BD audit	+X% lift	Within 6 mo	Providers

Owners listed by role; specific names to be assigned in BRD.

BLOCK 7 — PEOPLE

Key team:

Name	Role	Function
Jorge Sanchez Jr.	SFP Operator / SyncMD lead	Project sponsor, vendor selection, rollout owner
Dr. JSM Sanchez-Masiques	Physician-owner	Clinical champion, provider buy-in
Christopher	Rollout team (SFP)	Site-by-site rollout execution
Jessica	Rollout team (SFP)	Site-by-site rollout execution
Oriana Group (PRM)	Billing partner	Denial tracking, V28 QC, integration support
SyncMD / PRM dev team	Tech build	Module dev, dashboard integration

Accountable: Jorge Sanchez Jr.

Approvers: Dr. JSM Sanchez-Masiques + Jorge Sanchez Jr.

Locations:

- 5 SFP clinics: Coral Way, Hialeah, Pembroke Pines, Homestead, West Kendall
- **Off-site PCP offices** where SFP providers see patients in the PCP's own location

Scope flag: Off-site PCP coverage requires portable/mobile hardware and BAAs with each PCP office.

CEO update cadence: Sprint-based reporting. Jorge wants **user story hours estimate + sprint count**. Dashboard format preferred.

BLOCK 8 — RISKS & CONSTRAINTS

Top risks:

1. PRM dev team capacity — they build SyncMD, may be over-committed
2. Provider input burden — if rollout demands too much provider time, adoption stalls
3. Integration failure — vendor can't connect cleanly to SyncMD dashboard

Gut worry: Total elapsed time from start (today) to full rollout.

Constraints:

Constraint	Detail
Budget (one-time capex)	\$5K cash infusion for this module. Dev team labor already paid (existing SyncMD overhead, not project-attributable)
Ongoing AI vendor licensing	TBD — separate operating budget (likely \$2.4–4.8K/mo at industry per-seat rates)
Hard deadline	None — flexible
Resource limit	PRM dev team capacity
Technical limit	Must integrate with SyncMD dashboard + CureMD EMR
Compliance	HIPAA, BAA with AI vendor, V28 compliance built-in

Failure scenario: *"That's not a choice."* Executive commitment to outcome regardless of path.

BLOCK 9 — ROI

Quantified value (hard \$):

Value	Annual \$
MA OT eliminated	~\$93K
Sleep denials recovered	~\$15K
Provider retention (avoided cost)	~\$50K
Floor	~\$158K/yr

Qualitative value:

- V28/HCC compliance = PCP-facing competitive moat (HCC dollars flow to PCP/MSO under MA risk models, not SFP)
- Indirect lift: PCP retention + referral volume growth from "notes-available-immediately" positioning
- Strategic: SFP as cutting-edge pulmonary group → PE-readiness signal

Investment: \$5K one-time capex + TBD recurring AI vendor licensing.

Time to results: Full rollout deadline 2 months. First measurable improvement at pilot site within ~3–4 weeks.

BLOCK 10 — TIMELINE

Start: Within 2 weeks (~June 8, 2026).

Full rollout target: ~August 8, 2026 (flexible).

Phase	What happens	Target	Owner
1	Build-vs-Buy decision (PRM input on outsource vs. in-house) + vendor selection / internal scoping	Wk 1–2	Jorge Jr / PRM
2	SyncMD integration build — embed AI into SyncMD module	Wk 2–5	SyncMD / PRM dev
3	Pilot at small site (1–2 providers live)	Wk 5–6	Christopher / Jessica
4	Site-by-site rollout (Pembroke → Hialeah → Coral Way)	Wk 6–8	Christopher / Jessica
5	Off-site PCP coverage (mobile setup)	Wk 7–9	Christopher / Jessica
Go-live	All providers, all sites, all settings	~Aug 8, 2026	Jorge Jr

BLOCK 11 — PRIORITY SCORE

Question	Score
IMPACT	1 (Medium)
URGENCY	1 (Medium)

Question	Score
EFFORT	0 (Low-hard — multi-month, not a 2-week job)
TOTAL	2 / 6 → DELEGATE

Framework note: DELEGATE score reflects effort scale, not strategic importance. Sponsor has committed "failure is not a choice" + 2-month rollout target. BC author should flag that the rubric understates strategic importance for multi-month infrastructure builds.

FLAGS FOR BC AUTHOR

1. **Baselines to pull before finalizing BC Part 1:** note turnaround time (provider note completion dashboard), current sleep denial \$ trend, HCC capture rate (V28 audit).
2. **Budget gap:** \$5K capex does not cover ongoing AI vendor per-seat licensing — needs separate operating budget line.
3. **Timeline aggression:** 2-month full rollout is tight for vendor selection + integration build + 5 sites + off-site PCP. Stress-test against PRM dev capacity.
4. **HCC value reframe:** V28 compliance is a PCP-facing referral moat, not SFP coding revenue. Frame accordingly.
5. **Scope expansion:** Off-site PCP office coverage adds hardware/BAA/network complexity not present in pure SFP-controlled rollout.
6. **Provider champion:** Need to name a specific non-JSM provider as adoption champion to accelerate peer adoption.

NEXT STEPS

1. Paste this summary into **Business_Case_Prompt_Part1.md** framework → generates Brief + Executive Summary + Sections 1–3.
2. Paste Part 1 output into **Business_Case_Prompt_Part2.md** → generates Sections 4–12 + Appendices.
3. Combine outputs into final BC document.
4. After BC approval, hand off to BRD generation prompt for RACI, detailed phase plans, and resource breakdowns.